Referral packet F07: readable copy

Source: F07.pdf (fictional), 11 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

Page map

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1	Fax cover sheet	fax_cover	good	
2	CalAIM Community Supports authorization request	calaim_auth	good	
3	Patient face sheet	face_sheet	good	
4	Emergency department note	other	good	
5	History and Physical - page 1	h_and_p	good	
6	History and Physical - page 2	h_and_p	good	
7	CIWA-Ar flowsheet	nursing_note	good	
8	Hospitalist progress note	progress_note	good	
9	Social work note	social_work	good	
10	Labs and vitals	labs	good	
11	Medication list	med_list	good	

Fax cover sheet

Pacific Crest Hospital (fictional)
Case Management Department
FAX TRANSMITTAL
To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Lena Fischer, MSW
Phone: 555-0180
Date: 07/20/2026 18:30
Pages (including cover): 11
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]
Message
41M admitted for alcohol intoxication/withdrawal. 72 hr hold complete, CIWA now <8.
Referral form to follow (online portal down). Hoping for d/c 07/21 AM.
CONFIDENTIALITY NOTICE
This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Pacific Crest Hospital (fictional)

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CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST
Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal
Member
Member Name: Marcus Doe
DOB: 01/25/1985
Member ID: OCH-21175409
Referring Provider
Facility: Pacific Crest Hospital (fictional)
Clinical
Primary Diagnosis: Alcohol dependence with withdrawal, uncomplicated
ICD-10: F10.230
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 07/21/2026
Requested Days: 14
Admit Date: 07/17/2026
Clinical summary: detox completed, medically stable, independent ADLs.
Plan Use Only
Status: Pending review

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Page 2: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

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Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

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Member Name: Marcus Doe
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Referring Provider

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Clinical summary: detox completed, medically stable, independent ADLs.

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Pacific Crest Hospital (fictional) - PATIENT FACE SHEET
Patient: Marcus Doe
MRN: PC-4450961
Account #: 99450961
DOB: 01/25/1985
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)
Encounter
Admit Date/Time: 07/18/2026 09:45 (ED arrival 07/18/2026 02:10)
Admit Type: Inpatient
Unit/Bed: 3E-330
Attending: Owen Gallagher, MD
Insurance
Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self
Emergency Contact
None listed

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 3: Patient face sheet

face_sheet | orientation normal | legibility good

Pacific Crest Hospital (fictional) - PATIENT FACE SHEET

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Account #: 99450961
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Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
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Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 07/18/2026 09:45 (ED arrival 07/18/2026 02:10)
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Unit/Bed: 3E-330
Attending: Owen Gallagher, MD

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Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

Emergency department note

EMERGENCY DEPARTMENT PROVIDER NOTE
Arrival: 07/18/2026 02:10 via EMS (found down at bus bench, arousable)
HPI
41 y/o M, unhoused, h/o alcohol use disorder, brought in intoxicated. Pt slurring, admits drinking "a fifth" of vodka today. Daily drinker ~1 pint/day x 10 yrs. Hx withdrawal seizure 2023. No head strike per EMS.
Exam
T 97.6F HR 104 BP 146/92 RR 16 SpO2 96% RA
Somnolent but arousable, ataxic gait, nystagmus. No signs of head trauma.
Data
BAL 312 mg/dL (02:40)
Glucose 92, Na 135, K 3.3 L, CO2 22, AG 12
CT head non-contrast: no acute intracranial abnormality
ED Course / MDM
Acute alcohol intoxication. Monitored. By 08:00 BAL 168, pt tremulous, CIWA 12 -> lorazepam 2 mg PO.
High risk of complicated withdrawal (prior seizure) -> admit to medicine.
Signed: J. Ortega, MD 07/18/2026 08:50

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Page 4: Emergency department note

other | orientation normal | legibility good

EMERGENCY DEPARTMENT PROVIDER NOTE

Arrival: 07/18/2026 02:10 via EMS (found down at bus bench, arousable)

HPI

41 y/o M, unhoused, h/o alcohol use disorder, brought in intoxicated. Pt slurring, admits drinking "a fifth" of vodka today. Daily drinker ~1 pint/day x 10 yrs. Hx withdrawal seizure 2023. No head strike per EMS.

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-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL
Admit order: 07/18/2026 09:45
CC
Alcohol intoxication, now withdrawal
HPI
41 y/o M with severe alcohol use disorder, prior withdrawal seizure 2023, presented to ED 07/18 02:10 intoxicated (BAL 312). Developing withdrawal sx in ED. Last drink ~01:00 07/18. Has never completed residential treatment. Unhoused x 3 yrs, sleeps near Example Ave transit center.
PMH
- Alcohol use disorder, severe
- Alcohol withdrawal seizure (2023)
- Hypertension (untreated)
Meds PTA
None
Allergies
NKDA
Social
EtOH as above. Tobacco 1 ppd. Denies IVDU; occasional cannabis.

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 5: History and Physical - page 1

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Admit order: 07/18/2026 09:45

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41 y/o M with severe alcohol use disorder, prior withdrawal seizure 2023, presented to ED 07/18 02:10 intoxicated (BAL 312). Developing withdrawal sx in ED. Last drink ~01:00 07/18. Has never completed residential treatment. Unhoused x 3 yrs, sleeps near Example Ave transit center.

PMH

- Alcohol use disorder, severe
- Alcohol withdrawal seizure (2023)
- Hypertension (untreated)

Meds PTA

None

Allergies

NKDA

Social

EtOH as above. Tobacco 1 ppd. Denies IVDU; occasional cannabis.

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Exam

T 98.9F HR 108 BP 152/94 RR 18

Tremor bilat UE, diaphoretic, anxious, AOx4. No hallucinations reported.

Assessment/Plan

1. Alcohol withdrawal, high risk (prior seizure) - CIWA-Ar q4h-q8h, lorazepam per symptom-triggered protocol (CIWA >= 8), seizure precautions
2. Thiamine 100 mg IV daily x 3 days then PO, folic acid, MVI
3. Hypokalemia - K repletion
4. Alcoholic ketoacidosis? - BHB ordered
5. Addiction medicine + SW consult

Signed: Owen Gallagher, MD 07/18/2026 11:30

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 6: History and Physical - page 2

h_and_p | orientation normal | legibility good

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Signed: Owen Gallagher, MD 07/18/2026 11:30

-- FICTIONAL DEMO RECORD - not a real patient --

CIWA-Ar flowsheet

CIWA-Ar FLOWSHEET

Date/Time | CIWA-Ar | Lorazepam given | RN
07/18 08:00 (ED) | 12 | 2 mg PO | JO
07/18 14:00 | 16 | 2 mg PO | KB
07/18 22:00 | 18 | 2 mg PO | KB
07/19 06:00 | 13 | 1 mg PO | TR
07/19 14:00 | 11 | 1 mg PO | TR
07/19 22:00 | 9 | 1 mg PO | AL
07/20 04:00 | 8 | 1 mg PO (LAST DOSE) | AL
07/20 10:00 | 6 | none | TR
07/20 16:00 | 5 | none | TR
Protocol: lorazepam for CIWA >= 8. No seizure activity. No hallucinations documented.

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 7: CIWA-Ar flowsheet

nursing_note | orientation normal | legibility good

CIWA-Ar FLOWSHEET

Date/Time | CIWA-Ar | Lorazepam given | RN
07/18 08:00 (ED) | 12 | 2 mg PO | JO
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07/18 22:00 | 18 | 2 mg PO | KB
07/19 06:00 | 13 | 1 mg PO | TR
07/19 14:00 | 11 | 1 mg PO | TR
07/19 22:00 | 9 | 1 mg PO | AL
07/20 04:00 | 8 | 1 mg PO (LAST DOSE) | AL
07/20 10:00 | 6 | none | TR
07/20 16:00 | 5 | none | TR
Protocol: lorazepam for CIWA >= 8. No seizure activity. No hallucinations documented.

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#3
Date: 07/20/2026 17:00
S: Mild tremor, sleeping better, eating. Wants to leave tomorrow.
O: T 98.4F HR 88 BP 136/84 RR 16 SpO2 97% RA. Fine tremor at rest, no diaphoresis. AOx4.
CIWA 6 at 10:00, 5 at 16:00. Last lorazepam 07/20 04:00.
A/P: Alcohol withdrawal improving.
- Continue CIWA monitoring overnight; if CIWA remains <8 off benzodiazepines, d/c AM 07/21
- Thiamine/folate/MVI PO
- Started amlodipine 5 mg daily for HTN
- Naltrexone discussed, pt considering
Signed: Owen Gallagher, MD

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Page 8: Hospitalist progress note

progress_note | orientation normal | legibility good

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- Naltrexone discussed, pt considering

Signed: Owen Gallagher, MD

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Social work note

SOCIAL WORK / ADDICTION MEDICINE NOTE
Date: 07/20/2026 11:15
Pt engaged, AOx4, no SI/HI, no psychosis.
Offered residential treatment referral (Example Recovery Center, fictional) - pt DECLINED: "I'm not doing a program right now."
Pt states he will "try to cut back" and "maybe" go to outpatient group; did not commit to abstinence.
Outpatient IOP intake info given (phone 555-0185); no appointment scheduled.
Pt interested in recuperative care for housing. Explained most programs require sobriety.
Signed: Lena Fischer, MSW

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Page 9: Social work note

social_work | orientation normal | legibility good

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Labs and vitals

LABS

07/18 02:40 | BAL 312 mg/dL | Glu 92 | Na 135 | K 3.3 L | CO2 22 | AG 12
07/18 12:00 | Beta-hydroxybutyrate 0.3 (neg) | Mg 1.6 L | Phos 2.8
07/18 12:00 | AST 98 H | ALT 54 H | T bili 1.0 | Alb 3.8 | INR 1.0 | Lipase 40
07/20 05:00 | K 4.0 | Mg 2.0 | Cr 0.8 | Plt 142 L
07/18 MRSA nares PCR: Negative
07/20 06:00 SARS-CoV-2 rapid antigen: Negative
07/18 CXR: no acute disease
07/18 RUQ US: hepatic steatosis, no cirrhotic morphology, no ascites

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
07/19 14:00 | 98.8 | 98 | 144/90 | 18 | 97% | RA
07/19 22:00 | 98.6 | 94 | 140/88 | 16 | 97% | RA
07/20 04:00 | 98.7 | 92 | 138/86 | 16 | 96% | RA
07/20 10:00 | 98.3 | 86 | 134/82 | 16 | 97% | RA
07/20 16:00 | 98.4 | 88 | 136/84 | 16 | 97% | RA

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Page 10: Labs and vitals

labs | orientation normal | legibility good

LABS

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07/18 12:00 | Beta-hydroxybutyrate 0.3 (neg) | Mg 1.6 L | Phos 2.8
07/18 12:00 | AST 98 H | ALT 54 H | T bili 1.0 | Alb 3.8 | INR 1.0 | Lipase 40
07/20 05:00 | K 4.0 | Mg 2.0 | Cr 0.8 | Plt 142 L
07/18 MRSA nares PCR: Negative
07/20 06:00 SARS-CoV-2 rapid antigen: Negative
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VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
07/19 14:00 | 98.8 | 98 | 144/90 | 18 | 97% | RA
07/19 22:00 | 98.6 | 94 | 140/88 | 16 | 97% | RA
07/20 04:00 | 98.7 | 92 | 138/86 | 16 | 96% | RA
07/20 10:00 | 98.3 | 86 | 134/82 | 16 | 97% | RA
07/20 16:00 | 98.4 | 88 | 136/84 | 16 | 97% | RA

-- FICTIONAL DEMO RECORD - not a real patient --

Medication list

CURRENT / ANTICIPATED DISCHARGE MEDICATIONS
Patient: Marcus Doe MRN: PC-4450961
- Thiamine 100 mg PO daily
- Folic acid 1 mg PO daily
- Multivitamin 1 tab PO daily
- Amlodipine 5 mg PO daily (NEW 07/20)
- Nicotine patch 21 mg/24h daily
- Naltrexone 50 mg PO daily - PENDING pt decision

Inpatient only
- Lorazepam per CIWA protocol (last dose 07/20 04:00) - not for discharge

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Page 11: Medication list

med_list | orientation normal | legibility good

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